جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : نعناعة رفعت بسطا حنس

المحافظة : الجيزه **الحي / المدينة :** العجوزه **العنوان :** ش نارى جرجس ارض اللواء العجوزة الجيزة

رقم البطاقة : 28403120101229 **رقم التليفون :** 01279868779 **تاريخ تسجيل الإستمارة :** 20-10-2026

العمر(تاريخ الميلاد) : 12-03-1984 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي المقر بما فيه

التوقيع:(----------) **التوقيع:(----------)**

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام

التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

جوستافا روسي

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------

خطة العلاج : ----------

المستشفي : ----------**قسم :** ----------

جهة العلاج : ----------**نفقات :** ----------

ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: تعناعة رفعت بسطا حنس
National ID: 28403120101229

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


جمهورية مصر العربية
بطاقة تحقيق الشخصية
نعناعه
رفعت بسطا حنس
٣ش مارى جرجس -ارض اللواء
العجوزة - الجيزه
٢٨٤٠٣١٢٠١٠١٢٢٩
KU8595911


٢٨٤٠٣٩٢٠١٠١٢٢٩
ربه منزل
انثى مسيحية متزوجة
رافت ذكريا تامر حنس
البطاقة سارية حتى ٢٠٣١/١٠/٣٠

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : نضاعه رفعت بسطا حنس
Sex: Female
3- Medical number: 83220837
Old Med : 2360-2024

2-ID number:
Patient Age : 41 Y

Family history	Family History -ve	Phone	Phone 01279858779	BMI	BMI	REASON OF MDT	REASON OF MDT POST OPERATIVE
marital status	Marital Status MARRIED	NRS Score	NRS Score	GROUP	GROUP A	Date of MDT	Date Of MDT 24/10/2024
Co morbidity	Co Morbidity htn	Menopausal status	Menopausal Status PERIMENOPAUSAL	Initial STAGE	Initial STAGE		

Clinical data

Breast Imaging :
It LIQ lesion 3.1 x2 cm , 0.2 cm away from nipple
It LIQ multiple group of microcalcifications

Mets Workup :
ct cap:free

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC BX COMMENT It gii with focal dcis	8/8 ER COMMENT:	7/8 PR COMMENT:	Her 2 Neu COMMENT -ve	SISH COMMENT:	25

T	N	M
2	0	0

Sonomammography

PATIENT HISTORY
LT SM+SLNB:IDC GII with focal DCIS+focal necrosis
+ve microcalcification PT2(3CM)
LN0/8

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN
1- ADJUVANT TAXOTERE ENDOXAN X4
2- ZOLADEX FOR OVARIAN FUNCTION
3- ADJUVANT ZOLADEX AI

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY	RADIOLOGY	RADIOLOGY
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: نعناعه رفعت باسطه MR. : 7693	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	

نشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 41-year-old female with a history of left breast invasive ductal carcinoma (IDC) grade II, estrogen receptor positive (ER 8/8), progesterone receptor positive (PR 7/8), HER2 negative, Ki-67 25%. At initial surgery in September 2024, the patient underwent left breast lumpectomy with sentinel lymph node biopsy (SLNB) revealing 0/8 positive nodes. Pathology demonstrated IDC GII with focal DCIS, focal necrosis, and positive microcalcifications, with a tumor size of 2 cm (PT2). The postoperative plan included adjuvant chemotherapy with Taxotere and Endoxan for four cycles, followed by ovarian suppression with Zoladex, and subsequent aromatase inhibitor therapy with Femara plus Zoladex. The patient has been on Femara, calcium, and Zoladex since July 2025, with a planned continuation until October 2025.

The most recent oncology note indicates that the patient is currently receiving aromatase inhibitor therapy with Zoladex and Femara for a total of three months, with the next pharmacy visit scheduled one month from the current date.

The latest internal medicine note confirms that the patient continues Femara, Zoladex, and supportive care for an additional three months, with the next visit scheduled for 8 November 2026.

تحريرا في ,

التاريخ :	20-09-2026	الوقت :	

الطبيب المعالج : ايه مدحت بدوي محمد

يعتمد ,

Patient Name	: الأستاذه / نعناعه رفعت يسطا حنس	Request No.	: 13002162
Patient No.	: 28403120101229	Request Date	: 10/07/2024
Path No.	: SR-2357/24	Verification Date	: 16/07/2024
Sex	: Female	Financial Category	: اورام دار السلام - مباشره
Age	: 40 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast swelling.

Nature of Specimen: Tru-cut biopsy, collected at 10/7/2024.

Gross Examination:
- Multiple grayish white tissue cores up to 1.5 cm, totally submitted.

Microscopic Examination:
* Histological type: Invasive duct carcinoma with focal mucoid activity.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+3) Grade III.
* Ductal carcinoma in situ: Focally positive.
* Perineural invasion: Negative.
* Angioinvasion: Negative.

Diagnosis:
Left breast swelling, Tru-cut biopsy:
Invasive duct carcinoma, Grade III, with focal mucoid activity and focal DCIS.
For further IHC (ER, PR, Her2, Ki67).

Signature
Prof. Dr. Lubna Omar.
Dr. Esraa Magdy.

Verified by :

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / نعناعه رفعت بسطا حنس	Request No.	: 13003721
Patient No.	: 28403120101229	Request Date	: 26/09/2024
Path No.	: 3648/24	Verification Date	: 12/10/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 40 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, Up-Front.
Nature Specimen: Left simple mastectomy and SLNs.

Gross Examination:
* Procedure: Left simple mastectomy specimen measured 19x15x4 cm with covering skin ellipse measured 14x5 cm.
* Tumor extent: A greyish white tumor bed site.
* Size: 3.5x2.3x2cm
* Skin involvement: Unremarkable grossly.
* Distance from margins: 1.2 cm away from deep margin and 1.5cm away from the skin
* Lymph nodes number and size: Multiple fibrofatty tissue peice measured 2.5x2.5x2.5 entangling 8 LNs up to 2x1 cm.
* Rest of breast tissue show glistinning rubbery cut section.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1)
* Ductal carcinoma in situ: Focal (40%) (Commedo & Non commedo)
* Skin involvement: Free nipple and areola.
* Lymph node involvement: Free SLNs frozen (0/5), Free parrafine SLNs (0/3).
* Resection Margins: Free deep margin.
* Microcalcifications: Positive.
* Additional pathologic findings: Focal necrosis.

Frozen section results:
Free SLNs (0/5).

Diagnosis:
Left breast, simple mastectomy and SLNs Biopsy:

Verified by :
Dr. Sagda El-Kady

CAP ACCREDITED
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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / نعناعه رفعت بسطا حنس	Request No.	: 13003721
Patient No.	: 28403120101229	Request Date	: 26/09/2024
Path No.	: 3648/24	Verification Date	: 12/10/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 40 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Invasive duct carcinoma, Grade II with focal DCIS, Focal necrosis and psoitive microcalcification.
Free deep margin.
Free nipple and areola.
Free SLN (frozen) 0/5 , Free SLN (parrafine) 0/3
Total Nodal status : Free SLNs (0/8).
- Stage: pT2, pN0

Signature
Dr. Marwa Mahmoud.
Dr. Sagda Elkady.

Verified by :
Dr. Sagda El-Kady

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الاسم / السيدة مروة محمد يوسف حسن	Request No.	: 13002162
Patient No.	: 28403120101229	Request Date	: 10/07/2024
Path No.	: SR-2357/24	Verification Date	: 23/07/2024
Sex	: Female	Financial Category	: أورام ـ ثدي - ميكروب
Age	: 40 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast swelling.

Nature of Specimen: Tru-cut biopsy, collected at 10/7/2024.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- IPT (Positive control showed appropriate reactivity).
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Moderate nuclear reaction in 90% of tumor cells (score 7/8).
- Her2: No membrane reaction in the tumor cells (score 0).
- Ki67: 25% nuclear reaction of tumor cells.

Conclusion:

Left breast swelling, Tru-cut biopsy:

Invasive duct carcinoma, Grade III, with focal mucoid activity and focal DCIS.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- Her2: Negative (score 0).
- Ki67: Positive in 25% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1,

Verified by :

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	الأسكاء / نعامه رفعت يوسفا حنس		
Patient No.	28403120101229	Request No.	13002162
Path No.	SR-2357/24	Request Date	10/07/2024
Sex	Female	Verification Date	23/07/2024
Age	40 Y	Financial Category	برنامج دار السلام - مباشر
Referreing Doc.		Department	Out Patient
		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

>1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8..

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr. Lubna Omar.
Dr. Menna Nabil.

Verified by :

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
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Immunohistochemistry
[[DOCTYPE:IHC]]

PATIENT NAME: NIENAEUH REFAT BASTA HANES
GENDER: Female
DATE : 01-Jul-2024 08:55 AM

PATIENT ID: 10124244273
AGE: 40 y
ORDER: 2913395,2913396

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- Left breast mass.

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

- Heterogenous dense breasts which may obscure small lesions (ACR C).

Left breast:

- Left LIQ obscured lesion measures about 3.1x2.4 cm, 2.2 cm away from the nipple (measured in MLO view).
- Overlying left LIQ multiple groups of microcalcifications extending deeply, seen over an area of about 7x5 cm, about 1.7 cm away from the nipple (measured in CC view).
- Corresponding by US to left LIQ 7:00 ill defined hypoechoic lesion measures about 2.4x1.6 cm, 2.5 mm away from the overlying skin. Surrounding prominent ducts are seen reaching retroareolar and showing echogenic content. No vascularity detected on color Doppler application.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- No detected pathological axillary lymph nodes.
- No detected pathological supra or infraclavicular lymph nodes.

Right breast:

- No spiculated mass lesions or grouped microcalcifications.
- Complementary Ultrasound showed no solid or cystic mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Right nonspecific axillary lymph nodes.

OPINION:

Mammogram
[[DOCTYPE:MAMMOGRAM]]

breast malignant looking lesion with related suspicious LIQ multiple groups of microcalcifications for histopathological assessment (BIRADS 5).

- Bilateral mammary adenosis (BIRADS 2).

RECOMMENDATION:
Referral to clinic

Reported By,

Dr. Lamiaa Abdallah,MSc **01-Jul-2024 09:25 AM**

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Immunohistochemistry
[[DOCTYPE:IHC]]

Women's Health Outreach Program

Name: ne3naa Refaat yastta 40
Date: 3-7-2024

Full field Bilateral Digital Mammography & ultrasonography

Clinical summary left breast palpable lump from one week
Detailed Findings
Technique
Craniocaudally &Medio lateral oblique views
High resolution ultrasound
Left breast

- Dense glandular mammary parenchyma ACR c
- LIO regional micro calcifications seen in area about 7x2 cm away from nipple by 2 cm ,with underlying partially defined irregular round dense lesion measures about 2.5x2cm
- Complementary US corresponding to mammo finding between 8-9 o'clock shows irregular inhomogeneous hypoechoic solid lesion measures 30x20mm away from nipple by 2cm and away from skin by 2mm
- Multiple enlarged axillary nodes seen at level one with preserved fatty hilum and cortical thickening not excessed 3mm ,at level II enlarged lymph node measures 12x8mm with cortical thickening 4mm

Right breast

- Fairly dense glandular mammary parenchyma ACR density c
- No masses density ,asymmetry or suspicious clusters of microcalcifications.
- Ultrasound demonstrated mottling parenchymal appearance
- No solid or cystic lesion
- Preserved axillary LNs .
- **Conclusion:**

Left breast (BIRADS5) malignant looking solid lesions with regional micro calcification for biopsy
Right breast: (BIRADS 1).

DR Marwa M.Hassan

Mammogram
[[DOCTYPE:MAMMOGRAM]]

PATIENT NAME: NIENAEUH REFAT BASTA HANES
GENDER: Female
DATE : 31-Jul-2024 02:02 PM

PATIENT ID: 10124244273
AGE: 40 y
ORDER: 2968474,2968475

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH TRIPHASIC STUDY

TECHNIQUE:

- Multislice helical CT acquisition with automatic IV low osmolar non-ionic contrast medium infusion 50 ml.

FINDINGS:

1-CHEST

- Left breast lower inner quadrant irregular heterogenously enhancing mass lesion is seen measuring about 3.3x 2.7 cm along its maximum axial dimensions.
- No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- Two bilateral pulmonary small calcified nodules, likely post granulomatous sequel.
- No CT evidence of pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.
- Small hiatus hernia.

2- ABDOMEN AND PELVIS

- The liver is enlarged showing homogenous reduced parenchymal attenuation denoting fatty infiltration. No pathologically enhancing focal lesions noted throughout different study phases.
- Prominent common hepatic and common bile duct with CBD measuring about 8 mm. Slightly prominent pancreatic duct.
- No intra-hepatic biliary radicles dilatation.
- Patent homogenously enhancing portal vein and its main branches.
- Bilateral adnexal small cysts, for US correlation.
- Normal appearance of the pancreas, spleen, kidneys (right renal tiny gravel and few bilateral renal cortical cyst, the largest on the left),

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16602
CT Scan
[[DOCTYPE:CT_SCAN]]

- I.V.C and the rest of the pelvic organs.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No pathologically enlarged abdominal or pelvic lymph nodes.
- No abdominal or pelvic masses or collections.

The revised bone window revealed:

1. No CT evidence of definite osseous lytic or sclerotic lesion.
2. No CT evidence of vertebral collapse or extra-osseous soft tissue component.
3. Left femoral subcortical pseudocystic changes, likely degenerative.

Opinion:

- **Left breast malignant process, for histopathological correlation.**
- **No definite pulmonary or osseous metastatic deposits.**
- **Fatty hepatomegaly with no CT evidence of hepatic metastatic deposits.**

Reported By,
Dr. Nouran Atress,MSc
Dr. Mennatallah Mohamed, MD **31-Jul-2024 03:20 PM**

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CT Scan
[[DOCTYPE:CT_SCAN]]